Medicare payment per hospital stay rose 14% in two years

MASSACHUSETTS HOSPITALS | MEDICARE INPATIENT | 2019 vs 2021

2019 average

$16,075

per discharge

2021 average

$18,315

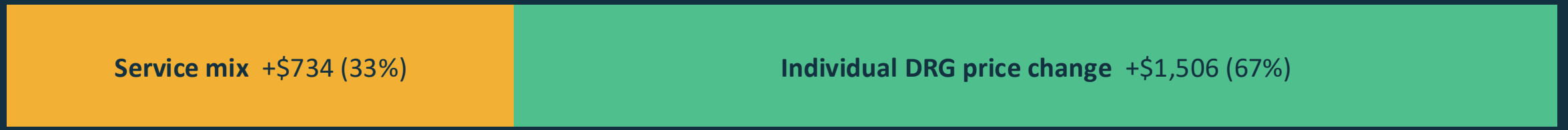
per discharge

Increase

+$2,240

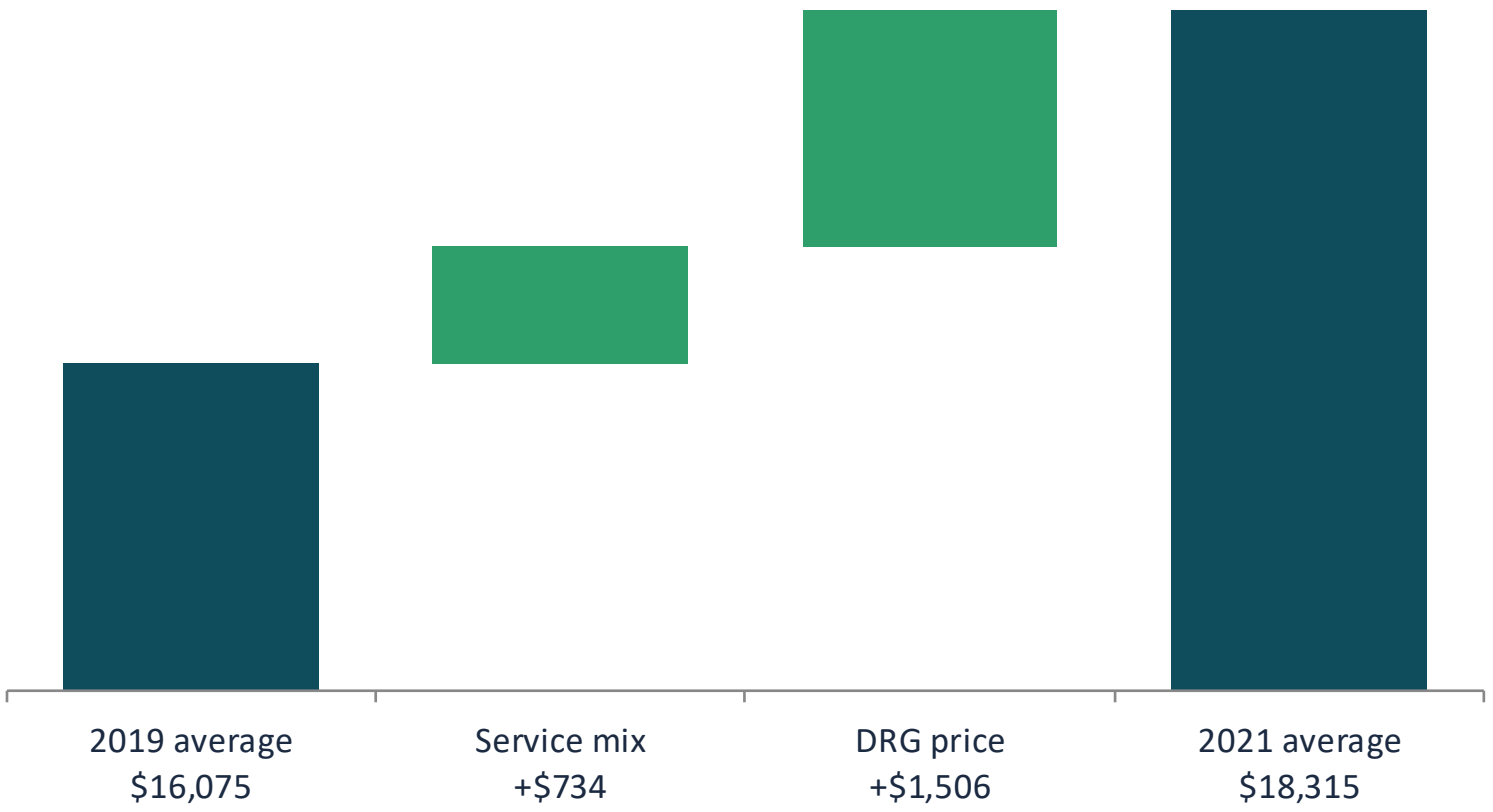
+13.9%

What drove the +$2,240?



Mix = hospitals treated a costlier blend of patients. Price = the same DRG was paid more per case.

Two-thirds of the rise came from higher DRG prices



+$734

Service mix (33%)

Hospitals shifted toward costlier patients, led by one respiratory DRG.

+$1,506

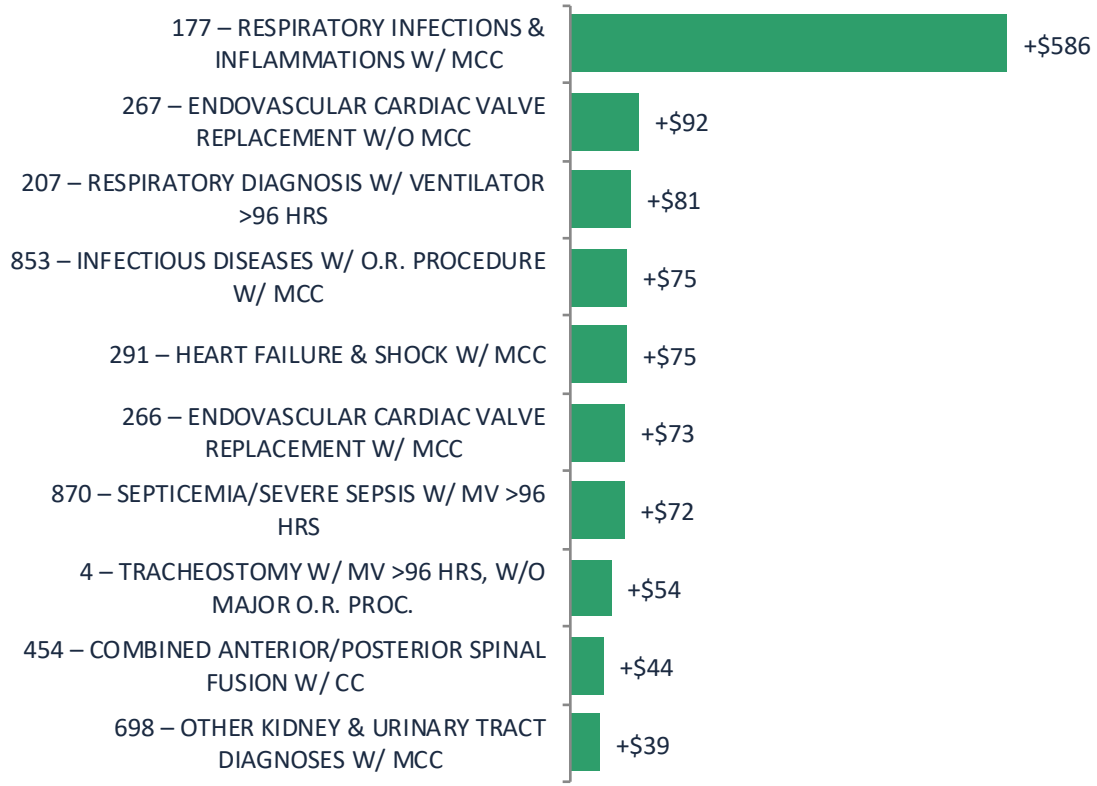
DRG price (67%)

The same DRGs were paid more per case, mostly sepsis and respiratory.

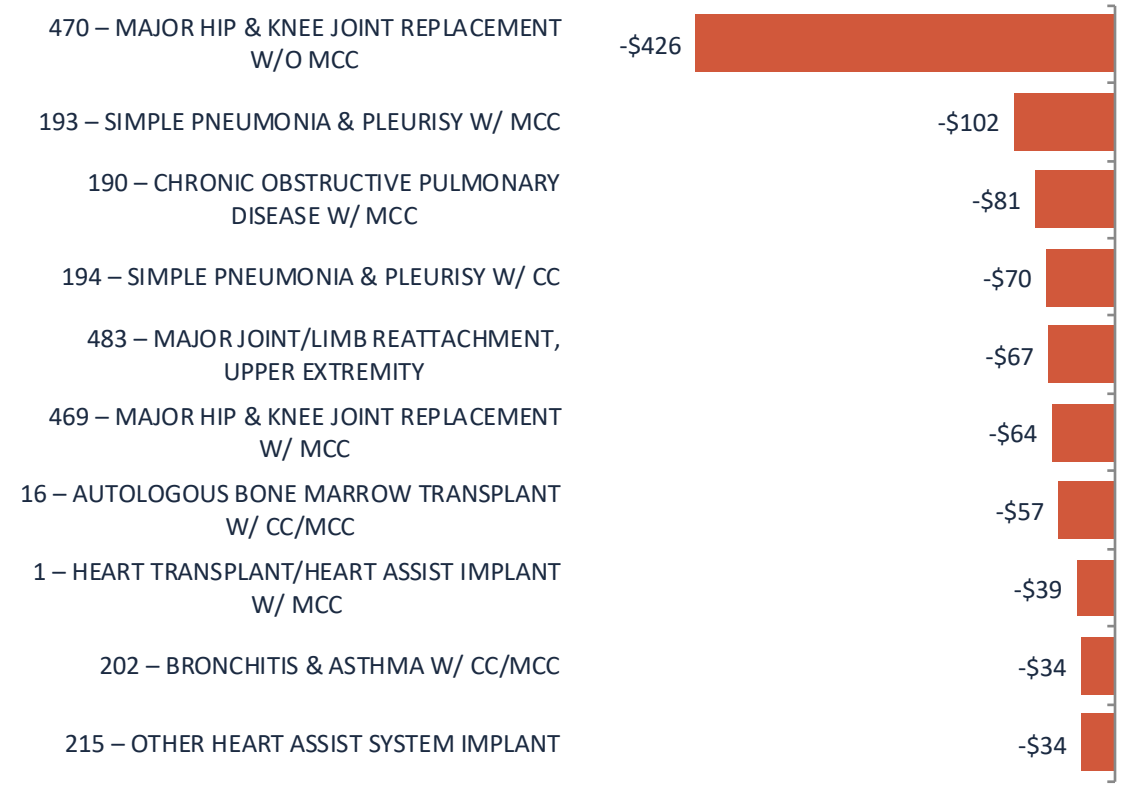
Axis starts at $14,000, not zero, so the two effects are visible.

Mix: one respiratory DRG did most of the lifting

Raised the average: top 10



Lowered the average: bottom 10

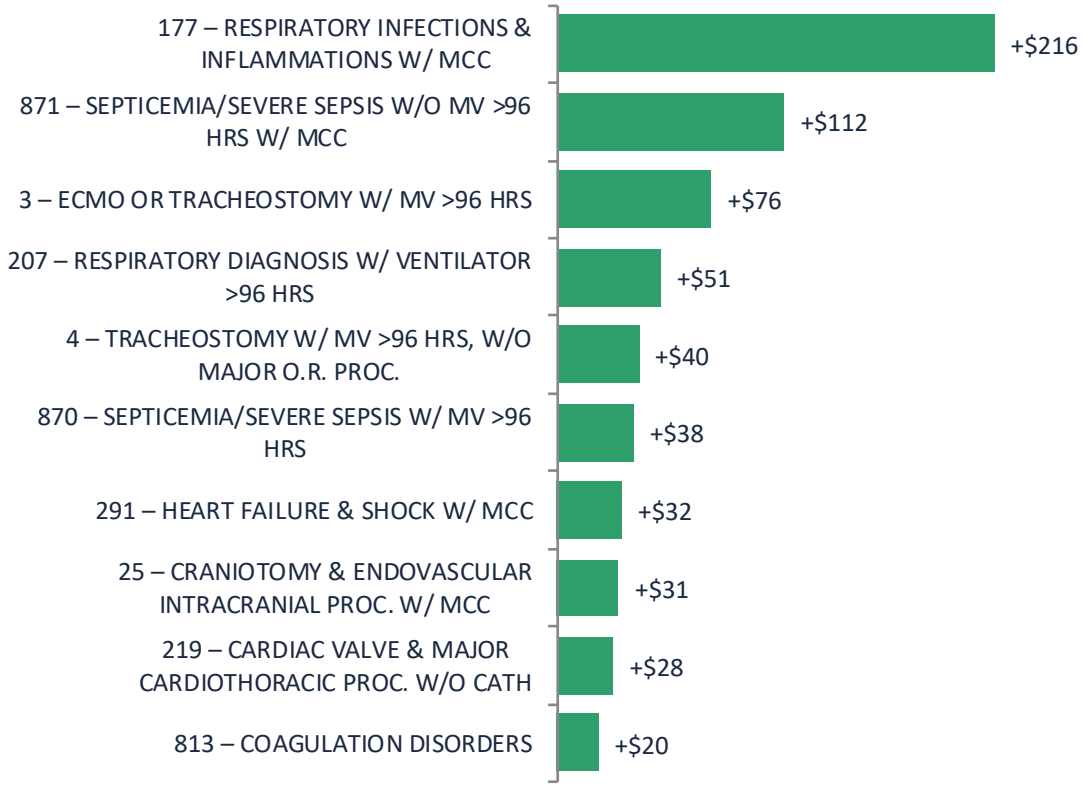


DRG 177 alone added $586 of the $734 mix effect (80%). Elective hip & knee replacements (470, 469) fell the most.

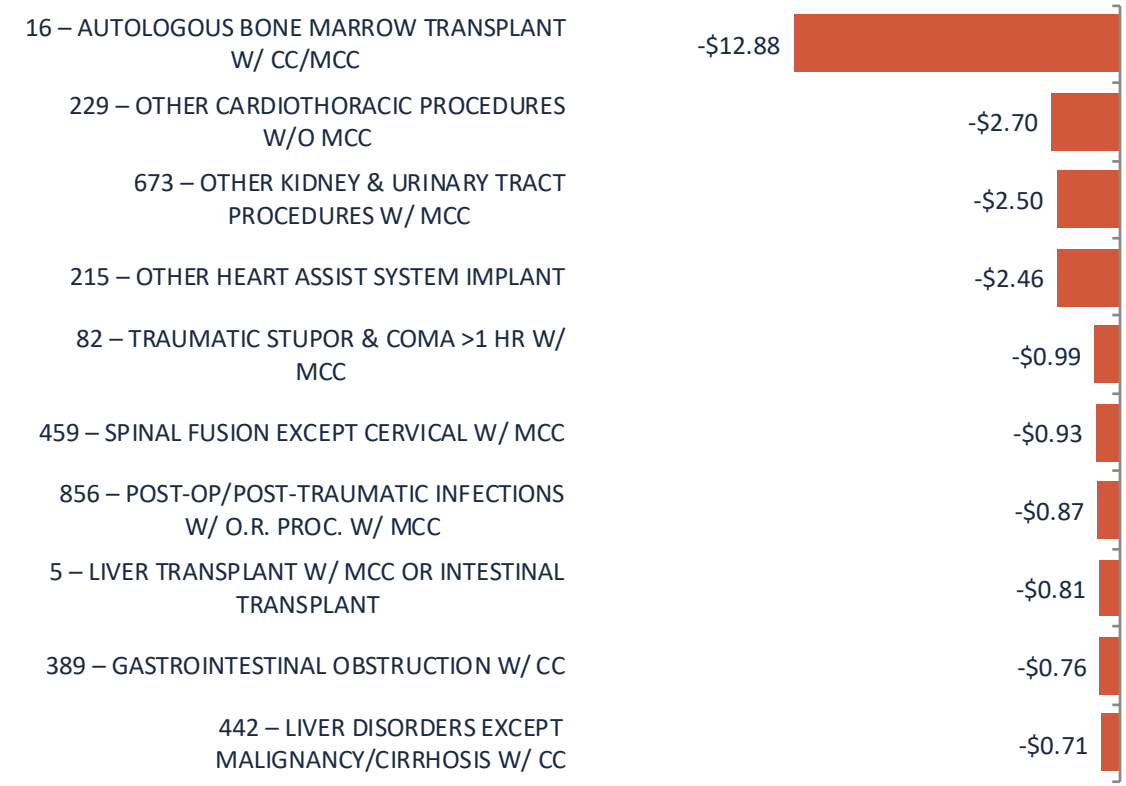
W/ = with, W/O = without. DRG names shortened to fit; codes unchanged. Only DRGs present in both years are included.

Price: a few sepsis and respiratory DRGs got paid much more

Raised the average: top 10



Lowered the average: bottom 10



Increases reach +$216 per discharge; the biggest decrease is only -$13. No broad price cuts.

W/ = with, W/O = without. DRG names shortened to fit; codes unchanged. Only DRGs present in both years are included.

So what for the acquisition

1

The rise is concentrated

DRG 177 is 80% of the mix effect and the top price driver. Five of the top 10 DRGs appear in both lists.

2

It looks COVID-shaped

More sick respiratory, sepsis and ventilator cases; fewer elective hip and knee replacements. 2021 may not repeat.

3

Next step in diligence

Check each target's exposure to these DRGs, and re-run with 2022 and later data before valuing 2021 payment per case.

Caveats: Medicare inpatient data for Massachusetts hospitals only; DRGs present in only one year are excluded; the COVID link is our interpretation of the DRG pattern, not tested.